emergency services to all'. Urban health services are to be expanded substantially from 'token interventions' to 'on-scale assured interventions' of 'primary healthcare delivery and referral support for urban poor'. This marks a massive shift to comprehensive public health services funded by the exchequer.

Comprehensive primary healthcare services would be delivered through 'Health and Wellness Centres' established on geographical and population norms to provide convenient access to every household. Secondary healthcare services would be expanded at the district and sub-district levels to provide specified secondary services like caesarean sections and neonatal care with the provision of at least two beds per 1000 population such that 'it is accessible within the golden hour'.

Public hospitals are to be repositioned as a tax-financed, single-payer healthcare system, where care is prepaid and cost-efficient, quality of care is imperative and facilities undergo periodic measurements and quality certification. Public hospitals would provide universal access to a progressively wide array of free drugs and diagnostics.

The Policy prioritizes the primary healthcare needs of the urban population with special focus on poor populations living in slums and other vulnerable populations such as the homeless, ragpickers, street children, sex workers and temporary migrants. It also seeks to achieve convergence among wider determinants of health, such as air pollution, solid waste management, water quality, occupational safety, vector control and urban stress.

It advocates resource allocation of a major proportion (up to two-thirds or more) to primary care, followed by secondary and tertiary care, and proposes some realignment in the financing of public healthcare facilities. Operational costs would be reimbursed for care provision and on a per-capita basis for primary care. Infrastructure, maintenance and salary expenditure of staff and administrative expenses will be funded on an